

1) IF — SHOULD IT BE TREATED?

Clot	Decision
Proximal DVT / main or segmental PE	Anticoagulate
Isolated distal (calf) DVT	Low-risk → repeat duplex 1–2 wk, treat if extends; treat now if severe sx/high risk
Isolated subsegmental PE, no proximal DVT	LE duplex; surveillance an option if low recurrence risk (CHEST 2021)

2) WHERE — INPATIENT vs OUTPATIENT

Risk-stratify PE with **PESI** / **sPESI**:

PESI <85 (I–II)	Outpatient OK if no high bleed risk, instability, or social barrier
PESI ≥85 / unstable	Inpatient (massive PE → thrombolysis + ICU)

Always screen the **social/logistic** factors (reliable follow-up, drug access) — as important as the score.

3) WHICH — AGENT

Agent	Bridge	Role
DOAC	No (apix/riva); dabig/edox need ~5d heparin	First-line for stable VTE incl most cancer VTE
LMWH	—	Pregnancy, poor PO absorption, luminal GI cancer, bridging
Warfarin	Yes	APS, mech valve, CrCl <25–30

Cancer VTE: DOAC > LMWH (CARAVAGGIO). **Luminal GI cancer** → **apixaban or LMWH** (GI bleed). **APS** → **warfarin, NOT DOAC** (TRAPS).

4) HOW LONG — DURATION

Scenario	Duration
Provoked (reversible cause)	~3 months
Unprovoked + high bleed risk	~3 months, then reassess
Unprovoked, not high bleed risk	Indefinite
Irreversible cause (active cancer, APS)	Indefinite (while cause persists)

Extended phase: reduced-dose DOAC — apixaban 2.5 mg BID (AMPLIFY-EXT) or rivaroxaban 10 mg daily (EINSTEIN-CHOICE) — cuts bleeding.

STARTING DOSES (CrCl-permitting)

Apixaban	10 mg BID ×7d → 5 mg BID
Rivaroxaban	15 mg BID ×21d → 20 mg daily (with food)
Edoxaban / Dabigatran	After ~5d parenteral heparin

WORK-UP & COUNSELING

- Baseline CBC, renal, LFTs; weight; assess bleeding risk & drug interactions
- Unprovoked → age-appropriate cancer screening; thrombophilia testing only if it changes management
- Counsel adherence, bleeding precautions, follow-up timing